

February 20, 2026

Office of the Deputy Secretary
Assistant Secretary for Technology Policy (ASTP) and the Office of the National Coordinator
for Health Information Technology (ONC)
U.S. Department of Health and Human Services
ATTN: Request for Information: HHS Health Sector AI RFI
Mary E. Switzer Building, Mail Stop 7033A
330 C Street SW
Washington, DC 20201

Submitted electronically via Regulations.gov

**RE: Request for Information (RFI): Accelerating the Adoption and Use of Artificial
Intelligence as Part of Clinical Care (RIN 0955-AA13; 90 FR 60108)**

Dear Deputy Secretary,

On behalf of the Massachusetts Health & Hospital Association (MHA) and our 64 acute care hospital and health system members, thank you for the opportunity to comment on HHS's Request for Information (RFI) on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care. MHA supports HHS' whole-of-department approach across regulation, reimbursement, and research and development, and offer below our recommendations organized to align with the RFI's ten specific questions.

Our members' input reflects broad, real-world experience implementing AI tools across clinical and operational workflows. Our members believe AI tools should adhere to "FAVES" principles (fair, appropriate, valid, effective, and safe). In particular, safety must be the guiding principle for AI adoption. Efficacy without safety is insufficient. We believe HHS should emphasize proportionate-to-risk safety, validation, transparency, and governance as co-equal aims with AI adoption and speed.

Executive Summary of Recommendations

MHA's members offer these high-level recommendations for HHS:

- HHS should lead with safety and direction through proportionate-to-risk frameworks and clinician-inclusive AI governance.

- HHS should fund and enable AI access with real-world evidence through grants, pilots, and demonstrations, especially for smaller, rural, and safety net providers.
- HHS should enable an evaluation and market infrastructure with standards-based certification, benchmarking, and interoperability.
- HHS should clarify liability, privacy, and disclosure expectations without creating burdensome mandates for healthcare providers.

MHA and our members offer these responses to the questions included in the RFI:

1. What are the biggest barriers to private sector innovation in AI for health care and its adoption and use in clinical care?

The greatest barriers to AI adoption and use in clinical care are safety, validation, and funding gaps for provider adoption as well as high early-stage costs with unclear return on investment; interoperability and integration frictions; the pace of change outstripping governance and workflow redesign; strategic misalignment between vendor offerings and health system priorities; and the risk of widening digital divides for smaller, rural, and safety net providers.

Additionally, Massachusetts hospitals mentioned specific concerns in each area, including:

- **Safety & Validation:** In particular, MHA members are concerned that there is insufficient real-world evidence that AI tools improve outcomes, particularly across diverse populations and care settings. Currently, there is overreliance on vendor test environments rather than local validation across real-world populations. More grant funding is needed to support co-development and conflict-free real-world validation studies to identify clinically appropriate use cases that improve patient outcomes.
- **Cost & ROI:** Many AI vendors charge significant per-use and subscription fees, with return on investment difficult to quantify at this time. For example, benefits such as reduced physician burnout and improved patient care due to innovations, improved care management, etc. are currently soft and difficult to capture in a meaningful way.
- **Integration & Interoperability:** There are currently limited or evolving APIs and competing products create friction and challenges for healthcare providers, with lack of integration into EHRs. Additionally, constrained IT and operational bandwidth slow appropriate AI governance with safe implementation and monitoring. It is essential that future AI innovations allow for integration and interoperability to be effective in improving patient outcomes and reducing administrative efficiencies and cost.

- **Operational Capacity & Speed:** MHA members are uneasy that rapid AI deployment risks over-promising/under-delivering –which erodes clinician trust if workflows, training, and guardrails lag.
- **Clinical & Operational Relevance:** Perhaps most notably, hospitals stated that AI tools may target vendor-addressable markets rather than hospitals’ highest-value clinical needs. In areas with high clinical and operational relevance, like ambient documentation, there has been rapid and strong adoption of AI.
- **Funding:** There is significant concern that resource constraints will mean that smaller community, safety net, and rural hospitals risk being left behind since AI adoption may flow to larger, urban, better-resourced healthcare providers.

2. What are the regulatory, payment, and programmatic priorities to incentivize the safe & effective use of AI in clinical care and why?

MHA and our members believe AI tools should adhere to FAVES principles (fair, appropriate, valid, effective and safe). From the regulatory perspective, for the use non-medical device clinical AI, we encourage HHS to establish voluntary, standards-based certification for AI-enabled health IT modules that emphasize transparency (intended use, training data provenance), lifecycle change logs, evaluation disclosures, and risk management. Consider a “light-touch” governance expectation within hospital Conditions of Participation for organizations that use AI – for example, an AI governance function including clinician leadership – without prescribing specific tools or metrics. MHA members also suggest that HHS provide HIPAA guidance clarifying BAAs, model-training boundaries, de-identification, and prohibited secondary uses. Continue information blocking guidance to support automation-friendly access to electronic health information while preserving privacy and security.

In terms of payment, MHA encourages the administration to launch Center for Medicare and Medicaid Innovation demonstrations that reward outcomes-validated AI use-cases (e.g., faster stroke/PE care, documentation efficiency linked to access) with equity adjustments for rural/safety-net providers. Explore bundled or add-on payments for validated AI assistance integral to interpretations / procedures, tied to transparent evidence standards and ongoing monitoring.

Among the programmatic recommendations, MHA encourages HHS to fund SBIR-style pilots, cooperative agreements, and Cooperative Research and Development Agreements (CRADAs) to generate generalizable, real-world evidence across diverse populations and sites, including the prioritization of community, rural, and safety-net settings. In addition, MHA members urge HHS to require EHR vendors to provide equal technical access to third party applications, as in some cases, large vendors are using their control of workflows and APIs to disadvantage third party apps and innovation.

3. What novel legal and implementation issues exist for non-medical devices, and what role, if any, should HHS play to help address them?

At present, there is little to no case law addressing liability for medical errors involving AI, and existing legal frameworks do not adequately account for AI's role in clinical decision-making. Current doctrines, including traditional product liability and medical malpractice standards, were not designed for situations in which clinical decisions are informed by complex algorithms that may operate beyond the full understanding of either physicians or patients. Absent clear standards, healthcare providers may be exposed to disproportionate liability risk for harms associated with poorly performing AI tools. This risk could lead to patient harm, discourage the use of beneficial technologies, limit innovation in clinical care, and increase malpractice insurance costs through higher premiums, deductibles, or restrictive policy exclusions related to AI use.

As AI systems become more advanced and personalized, they are increasingly positioned as meaningful contributors to medical decision-making. These technologies may analyze large volumes of individualized data – including medical records, diagnostic tests, imaging, and data from wearable devices – to inform diagnosis and treatment. While such capabilities hold significant promise for improving patient outcomes, they also introduce new risks. When an AI system produces an incorrect or misleading output, the consequences for patients can be serious, particularly if the error contributes to misdiagnosis or inappropriate treatment. A further challenge arises when physicians choose *not* to use AI tools, or when they override or disagree with AI-generated recommendations based on their clinical judgment and patient harm subsequently occurs. This scenario presents a legal dilemma with no clear precedent. Holding healthcare providers liable both for relying on AI and for exercising independent judgment against AI recommendations places them in an untenable position. In these circumstances, policymakers must consider where responsibility should properly lie.

Additionally, it is important that regulators ensure that developer accountability is built-in for AI product performance – especially for so-called “black box” AI systems where the internal logic driving outputs is not fully explainable to clinicians or patients. If a reasonable clinician could not have known – or had no reason to know – that an AI output was erroneous, and it was reasonable to rely on that output based on the developer's representations, the accountability for resulting harm to patients should rest with the AI developer.

MHA recommends that HHS consider and enable liability allocation among clinicians, institutions, and vendors, and the evolving standard of care (including when omission of proven AI could be considered substandard). MHA recommends that HHS work closely with and take its guidance from nationally-recognized clinical professional societies in formulating any guidance around specialty-based standards of care. In terms of clinician use of outside tools, MHA urges HHS to clarify institutional duties, clinician responsibility, and recommended governance controls. Regulatory oversight can help establish

performance expectations, safety requirements, and accountability mechanisms for AI developers, while giving healthcare providers greater confidence that approved tools meet appropriate standards. Without such standards, providers may be left shouldering the entirety of liability risk for AI-related harms, particularly when developers face limited accountability for product performance. This lack of oversight not only increases risks to patient safety but also creates additional liability exposure for healthcare providers who seek to responsibly integrate innovative technologies into care.

For continuous learning and model updates, it is important that regulators set expectations for material-update notifications, change logs, and re-validation requirements. And for privacy and secondary use, MHA encourages HHS to clarify HIPAA-consistent BAAs, de-identification, and restrictions on vendor model training using PHI/limited datasets. HHS can reduce uncertainty through guidance and safe harbors tied to risk-appropriate validation, monitoring, and transparency.

4. For non-medical devices, what are the most promising AI evaluation methods, metrics, robustness testing, and other workflow and human-centered evaluation methods for clinical care?

MHA members encourage the application of use-case-specific endpoints and metrics to evaluate AI against outcomes that match the clinical intent (e.g., door-to-needle, diagnostic accuracy, readmissions, patient-reported outcomes, patient experience; for ambient tools: documentation time, message turnaround, burnout), rather than general metrics. Additionally, there should be pre-deployment local validation across subgroups with human-factors testing, workflow simulation, and bias analysis. Post-deployment monitoring metrics should include safety incidents, drift, equity, override rates, and user experience; and require vendor model cards and site-level telemetry. HHS should fund grants/contracts for multi-site, real-world testbeds, benchmark registries, shared protocols, and prize competitions. They should reinstate/expand SBIR-like funding for paired developer-provider pilot projects to encourage technological innovation and commercialization of products.

5. How can HHS best support private-sector activities (e.g., accreditation, certification, testing, credentialing) to promote innovative and effective AI use in clinical care?

MHA recommends that HHS prioritize safety and endorse a neutral, standards-based certification and credentialing framework that verifies transparency, intended-use disclosures, update/change controls, and evaluation evidence. Regulators should encourage industry-driven, real-world validation over test environment only results, and support vendor and implementer credentialing for safety and lifecycle practices (e.g., change management, incident reporting, governance).

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality and help reduce costs?

AI in ambient documentation has experienced strong early adoption and impact on healthcare provider burden and “pajama time”, with promising retention/burnout improvements in the longer-term and favorable costs relative to human scribes. It is difficult to quantify hard savings beyond scribe substitution at the moment. In radiology, AI has clear promise for targeted tasks (e.g., pulmonary embolism and large vessel occlusion detection/triage), with mixed results in other areas.

Currently, AI integration and workflow are often the rate-limiting step in terms of widespread adoption. Generative AI indirect clinical decision-making is not broadly deployed at scale outside of research and pilot programs, and engaging prematurely without further peer-reviewed validation studies could pose serious safety risks. The near-term value of this is strongest in adjacent uses (summarization, drafting, inbox support) with clinician oversight.

As for high-potential areas where AI could see broader adoption and impact, language/medical interpretation tools, agentic population-health follow-up (e.g., readmissions outreach) and remote monitoring, and predictive analytics (e.g. for triage or identifying high-risk patients), evaluated for safety, equity, and patient-centered outcomes show promising outcomes in limited deployment. There is also significant opportunity to use AI tools for back-end billing and coding, automated claims denial responses, and informatics and interoperability functions such as mapping unstructured data to normative value sets, routing schema cleanup, and grouper and order set maintenance.

7. Which roles, decision-makers, or governing bodies within healthcare organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

At the hospital and health system level, those who are most influential with regards to the adoption of AI in clinical care include: AI governance committees; Chief Medical Information Officers (CMIO) / Chief Nursing Informatics Officers (CNIO) / Chief Information Officers (CIO); clinical department leaders; compliance/privacy staff; and quality/safety leaders.

When considering administrative hurdles, hospitals have indicated that there is limited operational bandwidth for the safe rollout of AI in clinical care, including training, change management, and appropriate post-implementation monitoring of tools. Hospitals also emphasized that leaders need to identify an AI strategy that is tied to their clinical priorities. They are concerned that there is a risk that immature deployments erode clinician trust. Moreover, the lack of hard ROI for most AI tools in clinical care today is a

barrier to adoption. This is a place where funding to subsidize the use of tools with a soft, yet unproven ROI and could help speed widespread adoption.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

MHA member hospitals and health systems recommend accelerating robust, scalable FHIR APIs and Digital Imaging and Communications in Medicine (DICOM) interoperability. There needs to be support for standardized access to notes, orders, images, and outcomes for evaluation. Additionally, there need to be appropriate safeguards around sharing data with vendors. In terms of benchmarking and data access, HHS should enable privacy-preserving data enclaves and de-identified datasets for external validation; support common telemetry schemas for monitoring and benchmarking real-world performance; and reduce friction in data-use agreements for lawful validation.

9. What challenges within healthcare do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Based on current use cases, patients recognize that AI deployment can lead to faster, safer diagnosis and treatment, in addition to improved overall access, enhanced language access, and more clinician engagement during patient encounters. Patients remain concerned with issues of privacy, particularly for sensitive health information, in addition to bias/fairness of algorithms and models used by AI. Additionally, patients want to have clarity about when AI is being used.

With regards to transparency for patients on provider use of AI, MHA and our member hospitals encourage the utilization and adoption of system-level notices (e.g., joint notice of information practices) stating AI may be used; require contextual disclosures/consent only where appropriate (e.g., two-party recording consent for ambient scribing; clear labeling for patient-facing chatbots). Healthcare providers would like to avoid per-use consent mandates that could let patients inadvertently opt in to lower-standard care as AI use becomes embedded as a standard practice.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

MHA and our member hospitals and health systems encourage HHS to prioritize AI safety and effectiveness “in the wild” using multi-site pragmatic trials with subgroup/equity analyses, human-factors outcomes, and longitudinal drift monitoring.

Additionally, HHS would be well-served with real-world economic evidence in support of the adoption of AI. There should be rigorous cost-effectiveness and ROI studies that link AI

to throughput, access, and downstream utilization—evidence that can support payment pathways for healthcare providers.

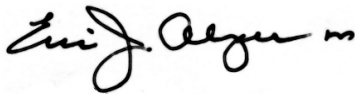
Finally, MHA and our members recommend prioritizing AI tools for research including: clinical language/translation; radiology and time-critical triage; population-health follow-up agents; and burden-reduction tools tied to measurable access and quality gains. We also recommend prioritizing AI tools for back-end informatics and interoperability functions such as mapping unstructured data to normative value sets, routing schema cleanup, and grouper and order set maintenance.

Thank you for your consideration of these comments. Collectively, MHA and our members look forward to engaging with HHS to protect patient safety and enable access to healthcare through the use of artificial intelligence. Should you have any questions or concerns, please reach out directly to Valerie Fleishman, Executive Vice President and Chief Innovation Officer, Massachusetts Health & Hospital Association at vfleishman@mhalink.org.

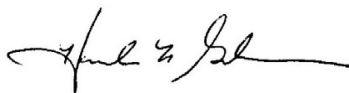
Sincerely,



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